

FUNDAMENTALS OF NURSING II

practical checklist



Medical surgical nursing team



Oral Medications Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Review the medical record for written physician order: Medication name, Dose, route, frequency. - Review the patient's medical history for any contraindications (allergies, conditions)			
2. Prepare Nurse: - Perform hand hygiene and put on PPE, if indicated.			
3. Prepare environment & equipment			
4. Prepare patient: Identify the patient & Explain procedure to the patient: Inform the patient about medication purpose, effect, possible side effects, and any special instructions (e.g., take with you a full glass of water, avoid crushing tablets).			
5. Ensure the patient is in a comfortable position (A sitting or upright position)			
6. Administer the medication and ensure they swallow it properly (offer water if needed). - For liquid medications, measure the exact dose with a medication cup or syringe. - For tablets instruct the patient that it should not be crushed - For powder medications use cup of water to dissolve medication in.			
7. Assess the patient for any adverse reactions (e.g., rash, swelling, dizziness) after the medication is taken.			
8. Document the administered medication: medication name, dose, time, and any relevant observations (e.g., the patient's response).			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Topical Medication Administration Checklist

Student name:

Date:

ID:

Procedure	Completely Done	Incompletely Done	Not Done
Review the medication order including the correct medication, dose, and site of application.			
Prepare patient: Identify the patient and explain the procedure to the patient			
Prepare environment			
Prepare nurse: Wash hands and apply gloves.			
Prepare Equipment the medication, gloves, any necessary applicators (e.g., cotton balls, gauze, or applicator sticks), and any other materials needed for proper administration (e.g., dressing for wound care).			
Assess the skin where the medication will be applied. Note any skin abnormalities, such as <u>redness, irritation, open wounds</u> , or other skin conditions that might affect the medication's effectiveness or absorption.			
If the skin is dirty, infected, or has an accumulation of ointment or debris, clean the area <u>with mild soap and water</u> .			
Apply a thin layer of medication with a cotton swab			
Rub in or allow absorption: If instructed, gently rub in the medication or allow it to absorb naturally (e.g., for patches, ointments, or lotions). - For certain medications, like creams, allow the medication to dry without covering it immediately.			
For transdermal patches , ensure the patch is securely affixed and remains in place.			

If instructed apply a sterile dressing (or bandage) or cover the area (if necessary) to the area (e.g., for wound care or burns).			
Observe the patient for any signs of side effects or adverse reactions , such as skin irritation, allergic reactions, or infection at the application site.			
Dispose of materials and remove gloves and wash hands thoroughly.			
Document the medication administration , including the medication name, dose, site of application, and any relevant observations (e.g., the patient's response or any side effects).			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Inhalation Medication Administration Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Review the medication order for the correct medication, dose, and inhalation device to be used.			
2. Prepare environment			
3. Prepare nurse: Wash hands and apply gloves.			
4. Prepare Equipment (e.g., spacer for metered-dose inhalers, mouthpiece, or mask for nebulizers).			
5. Prepare patient: Identify patient & Explain procedure to the patient.			
6. Position the patient in sitting upright posture. Ensure the patient's head is slightly tilted back (to open the airways) and that they are alert and able to follow instructions			
7. Instruct the patient to exhale fully to empty the lungs before inhaling the medication.			
8. <u>If using a metered-dose inhaler (MDI).</u> ➤ shake the inhaler to mix the medication. ➤ Instruct the patient to place the mouthpiece of the inhaler into their mouth or hold it about 1-2 inches away from their mouth. ➤ Ask the patient to inhale deeply and slowly while pressing the canister to release the medication. They should hold their breath for 5-10 seconds. - <u>If using a nebulizer.</u> ➤ Prepare the medication in the nebulizer cup according to the prescribed dose. ➤ Have the patient place the mouthpiece or mask over their mouth and nose. ➤ Turn on the nebulizer and ask the patient to breathe normally through the mouth until the nebulizer produces mist for about 5-10 minutes, or until the medication is fully delivered. ➤ Ensure the patient remains seated and avoids talking during treatment.			
9. Observe the patient during the procedure to ensure they are using the correct inhalation technique.			

10. Monitor for immediate adverse reactions to the medication, such as <u>coughing, wheezing, or difficulty breathing.</u>			
11. Rinse the mouth (if using a steroid inhaler)			
12. Provide post-inhalation instructions: Instruct the patient to avoid strenuous activity immediately after using the inhaler. wait at least 1-2 minutes between doses if more than one puff is prescribed.			
13. Clean the inhaler or nebulizer as directed and return it.			
14. Remove gloves and dispose of equipment appropriately			
15. Document the medication, dose, route, time of administration, and any observations or patient reactions in the patient's medical record.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor:

Student signature:

Sublingual Medication Administration Checklist

Student name:

Date:

ID:

steps	Completely Done	Incompletely Done	Not Done
1. Review the medication order including the correct medication, dose, and route of application.			
2. Prepare Nurse: Perform hand hygiene and put on PPE, if indicated.			
3. Prepare equipment Prescribed sublingual medication (e.g., tablet or film), gloves and pill cutter if required.			
4. Prepare patient: Identify patient & explain that the medication needs to be placed under the tongue and should not be swallowed.			
5. Position the patient in upright position.			
6. Administer the medication For tablets : place the sublingual tablet or film under the patient's tongue, and instruct the patient not to swallow or drink until it is fully dissolved (Typically for about 1-3 minutes) ○ For sprays : instruct the patient to place the mouthpiece under the tongue and spray once or twice as prescribed.			
7. Monitor for immediate reactions such as difficulty breathing, dizziness, or allergic reactions.			
8. Dispose of any waste properly			
9. Document the medication administered, including the drug name, dose, time, and any observations or patient reactions in the medical record.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Rectal Medication Administration Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Review the medical order (name, dose) and ensure the medication (e.g., suppository/enema) is at the appropriate room temperature before administration.			
Prepare Nurse: hand hygiene & apply gloves			
Prepare Equipment (suppository, enema, etc.), water-soluble lubricant, and tissues or a disposable pad.			
Prepare environment			
Position the patient in the Sims' position (left lateral recumbent) with knees flexed toward the chest, or the knee-chest position if required."			
Lubricate the tip of the suppository or enema nozzle with a small amount of water-soluble lubricant (never use petroleum jelly as it can interfere with absorption)			
Administer the medication <u>Suppository:</u> Gently insert the rounded end of the suppository into the rectum, about 1 to 2 inches for adults, past the internal anal sphincter. <u>Enema:</u> Gently insert the enema nozzle into the rectum, about 2 to 4 inches for adults. If administering the enema solution, slowly squeeze the bottle or use a syringe to release the liquid.			
Ask the patient to retain the medication for at least 15-30 minutes - For an enema, the patient may need to hold the solution for several minutes.			
Observe the patient for any discomfort, signs of complications (e.g., bleeding, pain), or adverse reactions.			
Clean up the area if necessary & Disposing of any used equipment			
Hand hygiene			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Administering Vaginal Suppositories Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Review the medication order			
Prepare nurse: hand hygiene & apply gloves			
Prepare patient: Identify patient, introduce yourself and explain the procedure			
Prepare equipment			
Prepare environment			
Place patient in dorsal recumbent or side-lying position.			
Remove foil wrapper from suppository & Insert into applicator.			
Lubricate applicator with lubricant to reduce friction.			
Insert applicator with suppository into the vaginal canal at least 2 inches to prevent suppository from slipping out.			
Instruct the patient to lie quietly for a minimum of 5 to 10 minutes until the suppository is absorbed.			
Discard equipment, wash applicator, and return to appropriate place in patient's room.			
Remove gloves and perform hand hygiene.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Ophthalmic Medication Administration Checklist

Student name:

Date:

ID:

Clinical instructor

Steps	Completely Done	Incompletely Done	Not Done
Review the medication order to verify the prescription (medication, dose) and ensure the medication is at the appropriate room temperature before administration.			
Prepare nurse: hand hygiene & apply gloves if needed			
Prepare patient: Identify patient, introduce yourself and explain the procedure			
Prepare equipment: the prescribed ophthalmic medication (eye drops or ointment), clean tissue or gauze, gloves (if required)			
Prepare environment			
Place the patient in sitting position with the head tilted slightly back and supported, looking upward.			
- For eye drops: Shake the bottle gently (if needed) and remove the cap. For eye ointment, squeeze out a small amount directly from the tube.			
Administer the medication <u>For Eye Drops:</u> Gently pull the lower eyelid to create a pocket, instill the drops without touching the eye or lashes to prevent contamination <u>For Eye Ointment:</u> Squeeze a small ribbon of ointment (approximately 1–2 cm) inside the lower eyelid, from the inner corner to the outer corner.			
"Immediately after instilling the medication, apply gentle pressure to the inner corner of the eye (nasolacrimal duct) for 1–2 minutes, then instruct the patient to close their eye gently and move it to distribute the medication."			
Wipe away excess medication with a clean tissue or gauze.			
Monitor the patient for any immediate reactions, such as redness, irritation, discomfort and ensure they understand how to use the medication for future doses.			
Dispose of materials and wash hands			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Nasal Medication Administration Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Review the medication order to verify the prescription (medication, dose)			
Prepare nurse: hand hygiene & apply gloves if needed			
Prepare patient: Identify patient, introduce yourself and explain the procedure			
Prepare equipment: (nasal spray or drops), tissues			
Prepare environment			
Have the patient sit up straight with their head slightly tilted forward.			
Shake the nasal spray bottle (if required). Remove the cap and prime the spray (by spraying once or twice into the air)			
Administer the medication • For nasal spray: Instruct the patient to tilt their head forward, insert the nozzle into one nostril, and close the opposite nostril and have the patient breathe in through the nose as the spray is released. Repeat for the other nostril, if necessary • For nasal drops: "Tilt the head backward, instill the drops into each nostril, and ask the patient to remain in this position for a few minutes.			
Encourage normal breathing and instruct the patient to avoid blowing their nose for several minutes after administration.			
Wipe any excess medication around the nostrils using tissue.			
Monitor the patient for any immediate reactions (such as irritation or allergic reactions) and ensure they understand how to use the medication for future doses.			
Dispose of materials and wash hands			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Transdermal Medication Checklist

Steps	Completely Done	Incompletely Done	Not Done
Review the medication order to verify the drug name, dose, and frequency, and assess the patient's history for any allergies or contraindications			
Prepare nurse: hand hygiene & apply gloves if needed			
Prepare patient: Identify patient, introduce yourself and explain the procedure			
Prepare equipment: gloves, alcohol wipes, the prescribed transdermal patch			
Prepare environment			
Select and prepare the application site - Choose a clean, dry, hairless area (e.g., upper arm, chest, back, or abdomen). - Assess the skin for irritation, cuts, or rashes at the intended site. - Rotate sites with each application to prevent skin irritation.			
Remove the old patch (if applicable) and fold it in half (sticky sides together) before disposal.			
Clean the application area with an alcohol swab or soap and water, then let it dry.			
Apply the patch: Peel off the protective backing and press the patch firmly onto the skin for 10–30 seconds without touching the adhesive side."			
Instruct the patient to avoid applying heat near the patch, never cut the patch, and report any side effects immediately.			
Dispose of used materials properly and perform hand hygiene.			
Record the date, time, application site, and any skin observations; initial the patch if required by institution.			

Student name:

Date:

ID:

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Instilling Otic Drops Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Review the medication order			
Prepare nurse: hand hygiene & apply gloves if needed			
Prepare patient: Identify patient, introduce yourself and explain the procedure			
Prepare equipment			
Prepare environment			
Position patient on unaffected side, with ear to be treated in the upper most position.			
Fill medication dropper with prescribed amount of medication.			
Lift pinna upward and backward.			
Instill medication drops, holding dropper slightly above ear.			
Instruct the patient to remain on side for 5 minutes after instillation.			
Dispose of gloves and perform hand hygiene.			
Documentation			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Intradermal Injection Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Check right name of medication, dose, expiration date & inspect for any discoloration			
Prepare nurse: Perform hand hygiene and apply gloves			
Prepare equipment: tray, paper bag, syringe, dry cotton, alcohol swab & clean gloves			
Prepare patient: Identify patient and explain procedure			
Draw the correct dose into the syringe. Ensure that there are no air bubbles by tapping the syringe and expelling the air.			
Assist patient into comfortable position.			
Select an accessible, hairless, and lesion-free site on the inner forearm (3–4 fingers below elbow/one hand above wrist), upper back (between/below scapulae), or upper chest.			
Use an antiseptic swab in a circular motion to clean skin			
<u>Administer injection:</u> - With non-dominant hand, stretch skin over site. - Insert needle slowly at a 5 to 15 angle bevel up. The needle tip should be seen through the skin. - Do not aspirate. Slowly inject the medication then, Withdraw the needle. Do not massage the site			
Note a small bleb, like a mosquito bite, forming under the skin surface			
Draw circle around the site and avoid scratch the site, wait for 20-30 min. - For tuberculin testing, instruct patient to return and have the site checked in 48–72 hr.			
Check for localized redness, itching, or signs of anaphylaxis.			
Discard the uncapped syringe in a safety box. Don't recap.			
Remove gloves and wash hands			
Document date and time of skin reaction and any systemic side effects of the medication (redness, swelling, severe pain). Report to physician			

Total grade:

Clinical instructor

Studentsignature:

Subcutaneous Injection Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Verify the Medication Order for correct medication, dose, and route (subcutaneous). Double-check the medication's expiration date			
Prepare Equipment: Medication (vial, ampule, or pre-filled syringe), Syringe (typically 1-3 mL), Needle, Alcohol swabs, Gloves, Bandage or gauze pad (optional)			
Prepare nurse: Perform hand hygiene and apply gloves			
Prepare patient: Identify patient and explain procedure			
Draw the correct dose into the syringe. Ensure that there are no air bubbles by tapping the syringe and expelling the air.			
Assist patient into comfortable position.			
Select an appropriate site ○ Abdomen: 2 inches away from the navel (for insulin, heparin). ○ Outer thigh: For larger volume injections in some cases. ○ Upper arm: For smaller injections.			
Clean the injection site with an alcohol swab, moving in a circular motion from the center outward.			
Hold the syringe like a pencil or dart.			
Use your non-dominant hand to pinch a fold of skin around the selected injection site for thin patients or stretch it for obese patients			
Insert the needle at a 45 to 90 angle (depending on the patient's body size and the length of the needle) into the pinched skin.			
Once the needle is in place, slowly push the plunger of the syringe to administer the medication.			
Remove the needle quickly. Apply gentle pressure to the site with a clean gauze pad or cotton ball.			
Observe the patient for any immediate reactions or complications (e.g., swelling, redness, allergic reactions).			
Dispose of the Needle and Syringe, and other disposable materials in an appropriate sharp container.			
Record the medication name, dose, route (subcutaneous), time, and site of injection, any adverse reactions or complications			

Total grade:

Clinical instructor

Student signature:

Intramuscular Injection Checklist

Student name:

Date:

ID:

steps	Completely Done	Incompletely Done	Not Done
Review the patient's medical order to ensure the correct medication, dose, and route (IM) are specified.			
Prepare equipment: (vial, ampule, or pre-filled syringe), syringe (appropriate size), needle, alcohol swabs, gloves, and a sterile gauze pad or bandage.			
Prepare nurse: Perform hand hygiene and apply gloves			
Prepare patient: Identify patient and explain procedure			
Withdraw the correct dose into the syringe. Ensure that there are no air bubbles by tapping the syringe and expelling the air.			
Assist patient into comfortable position.			
Select an appropriate site Deltoid (Upper Arm) Site: Two finger-widths below the acromion process. Use: Small volumes (up to 1 mL) for vaccines or rapid absorption. Vastus Lateralis (Outer Thigh) Site: Middle third of the outer side of the thigh. Use: Larger volumes (1–4 mL); safe for all ages, especially infants. Ventrogluteal “Gluteus Medius (upper buttocks)” Use: for larger volumes in adults (1–3 mL). Site: Center of the V-shape between the iliac spine and crest. Rectus Femoris (Anterior Thigh) Use: for larger volumes in adults (1–3 mL). Site: Middle third of the anterior (front) aspect of the thigh.			
Use an alcohol swab to clean the injection site starting from the center and moving outward in a circular motion.			

Hold the syringe like a dart, insert at 90°, Stabilize the syringe with your non-dominant hand, and aspirate for blood; if blood appears, remove and restart at a new site.			
Administer medication slowly and steadily, particularly for viscous solutions.			
Remove the needle quickly and apply gentle pressure to the injection site with a clean gauze pad or cotton ball.			
Dispose of the needle, syringe, and other waste materials in a proper sharp container.			
Observe the patient for any immediate reactions or adverse effects, such as pain, swelling, redness, or signs of an allergic reaction.			
Record the medication name, dose, route, injection site, time, and any relevant observations (e.g., patient's response, any adverse effects).			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Intravenous push Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Review medications record for medications to be given.			
Prepare equipment. Tourniquet, alcohol swab, adhesive tape, prescribed medications or labeled blood collection tubes (for sampling), along with appropriate-sized syringes and needles. Medication card, Clean gloves, Sterile gauze or transparent dressing • The Medication Card (MAR - Medication Administration Record) include: Patient's Full Name, Patient Identification Number: (e.g., Medical Record Number MRN or Bed Number), Medication Name, Dosage, Route, Frequency and Time, Expiry Date, Doctor's Signature: To verify the order is official.			
Prepare nurse: Perform hand hygiene and apply gloves			
Prepare patient: Introduce yourself, identify patient and explain procedure			
Assist patient into comfortable position.			
Withdraw the medication in a syringe			
Select venipuncture site: The vein should be palpable, straight and spongy.			
Apply Tourniquet around arm 10 to 15cm above the selected site			

Instruct the patient to open and close their fist several times, then keep it closed, while lowering and stroking the arm			
Disinfect the site with alcohol swab			
Stretch skin taut and stabilize vein with a non-dominant hand.			
Remove the needle cover and hold the syringe bevel up.			
Insert the needle to the skin at 30-degree angle.			
Aspirate for blood return through syringe backflow chamber.			
Release the tourniquet once blood flashback is observed.			
Inject the medication smoothly and gently.			
Assess for swelling, bruises, or burning.			
Withdraw the needle and apply pressure to the injection site using a dry cotton sponge.			
Don't recap the needle & discard the syringe in the sharp container			
Wash hands & Documentation: date and time, skin reaction and any systemic side effects of the medication (redness, swelling, severe pain) & Report to physician			
N.B In case of sampling: - Use sterile syringe and follow the same steps - Then withdraw the required dose.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Oxygenation Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Verify the Medical Order for oxygen therapy including flow rate (L/min), delivery device, and duration.			
2. Assess the Patient: oxygen saturation (SpO ₂) using pulse oximetry, signs of hypoxia (e.g., cyanosis, confusion, tachypnea), the respiratory rate, depth, and effort.			
3. Perform Hand Hygiene and Wear Gloves if needed			
4. Gather Supplies: appropriate oxygen delivery device (e.g., nasal cannula, face mask) and oxygen source.			
5. Explain the Procedure to the Patient			
6. Place the patient in a semi-Fowler's or high-Fowler's position (sitting upright).			
7. Connect and Set Up the Oxygen Device: - Attach the oxygen tubing to the oxygen source. - Adjust the flow rate as prescribed : ➤ Nasal cannula: 1-6 L/min (low flow) ➤ Simple face mask: 5-10 L/min ➤ Non-rebreather mask: 10-15 L/min (high concentration) - Ensure oxygen is flowing before applying the device			
8. Apply the Oxygen Device • <u>For a nasal cannula:</u> Place prongs into nostrils and loop tubing over ears. • <u>For a face mask:</u> Position over nose and mouth, securing straps.			
9. Initial monitoring of the Patient's Response • Reassess oxygen saturation (SpO₂) within 5-10 minutes. • Observe for adverse effects (e.g., oxygen			

toxicity, CO ₂ retention in COPD patients).			
10. Periodically monitor oxygen saturation and respiratory status to determine if adjustments are needed.			
11. Document the Procedure: Date and time of oxygen initiation, Oxygen flow rate and delivery method, oxygen saturation levels & Any adverse reactions			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Nasogastric tube Insertion Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Verify doctor's orders for NG tube type, reason for placement, and connection method (suction or drainage bag).			
2. Perform hand hygiene and gather supplies.			
3. Assess patient's level of consciousness and understanding of procedure.			
4. Visually inspect condition of patient's nasal and oral cavities.			
5. Assess for the best nostril before you begin. - Ask the patient about previous injuries or history of a deviated septum. - Test Airflow: Occlude one nostril at a time and ask the patient to sniff to find the best side.			
6. Palpate patient's abdomen for distension, pain, and/or rigidity & Auscultate for bowel sounds.			
7. Position patient in high fowler's at 45 to 90 degrees (unless contraindicated), with a pillow under the head and shoulders.			
8. Agree on a signal the patient can use if they wish you to pause during the procedure.			
9. Stand on the side of your dominant hand (Right/Left) for better control.			
10. Place a towel on the patient's chest and provide facial tissues and an emesis basin.			
11. Measure distance of the tube from - The tip of the nose to the earlobe - The earlobe to the xiphoid process and then marks the tube at this point.			
12. Curve 10 to 15 cm of the end of the NG tube around your gloved finger, and then release it.			
13. Lubricate NG tube tip according to agency policy.			
14. Provide patient with drinking water with a straw if the patient is not fluid restricted.			

15. Have patient drop head forward and breathe through the mouth.			
16. Insert NG tube slowly along the bottom of the nasal passage, with the curved end pointing downward toward the ear on the same side as the nostril.			
17. If slight resistance occurs, twist the tube and apply gentle downward pressure. If significant resistance is felt, remove the tube and try the other nostril.			
18. If there is difficulty in passing the NG tube, ask the patient to sip water slowly through a straw. If oral fluids are not allowed, ask the patient to try dry swallowing while you advance the tube.			
19. Continue to advance NG tube until you reach the mark/tape you had placed for measurement.			
20. Temporarily anchor the tube to patient's cheek with a piece of tape until you can check for correct placement.			
21. Verify tube placement using pH paper (acidic check) as an initial step to confirm that acidic contents are present, followed by an X-ray for final confirmation before feeding.			
22. Once the tube placement has been confirmed, mark (with a permanent marker) and record the length of tubing extending from the nose to the outer end of the tube.			
23. Secure the tube to the patient's gown with a safety pin, allowing enough tube length for comfortable head movement.			
27. Document the procedure according to agency policy and report any unexpected findings.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Urinary catheter Insertion ‘Male’ Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Assess order for catheterization.			
If catheter is to be changed, determine size previously used.			
Hand hygiene & Gather supplies.			
Provide privacy & ensure adequate lightening.			
Identify the client & explain procedure to patient			
Position the patient on his back with legs slightly apart and lying as flat as possible.			
Place waterproof pad under patient exposing only perineal area.			
Hand hygiene and apply sterile gloves.			
Prepare a sterile field.			
Organize equipment in sterile field.			
Attach the catheter to the collecting bag.			
Lubricate catheter tip by dipping it into water-soluble gel 12.5 to 17.5 cm (5 to 7 inches).			
Cleans urethral meatus: Hold the penis with a sterile swab and clean the penis thoroughly. Retract foreskin and clean the urethral meatus thoroughly using sterile swabs in a circular motion.			
Hold the penis vertically and insert the catheter into the urethra			
Check for urine flow before inflating the balloon			
Inflate the balloon using 10 ml of saline, ensuring that it does not cause any pain. N.B the volume used to fill the balloon may vary depending upon the size of the catheter used.			
Gently pull on the catheter until resistance is felt. Then reposition the foreskin.			
Attach drainage bag to bed frame with hook or strap			

provided.			
Anchor the catheter using non allergic tape. Tape to inside the thigh and allow some in catheter length.			
Coil excessive drainage tubing and place beside the client, the remaining tubing should run in a vertical line down to the bottom sheet with tape or clip provided with drainage set.			
Place drainage bag below level of bladder.			
Dispose of gloves, equipment and wash hands.			
Help patient adjust position.			
Evaluate catheter function and amount, color, odor and other urine characteristics.			
Document size of catheter inserted, amount of water in ballon, patient tolerance, and assessment of urine.			

If the student doesn't excess 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Urinary catheter Insertion “Female” Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Assess order for catheterization.			
If catheter is to be changed, determine size previously used.			
Hand hygiene & Gather supplies.			
Provide privacy & ensure adequate lightening.			
Identify the client & explain procedure to patient			
Assist patient to supine position with leg spread and feet together or side lying position with upper leg flexed			
Place waterproof pad under patient exposing only perineal area.			
Hand hygiene and apply sterile gloves.			
Prepare a sterile field.			
Organize equipment in sterile field.			
Attach the catheter to the collecting bag.			
Lubricate catheter tip by dipping it into water-soluble gel 2.5 to 5 cm (1 to 2 inches) for women.			
Separate labia with non-dominant hand to expose the meatus (keep hand in place until catheter insertion).			
Clean from meatus to anus using sterile cotton balls held by forceps: Use a new cotton ball for each side (labia minora), then straight down the meatus.			
Gently insert the catheter into meatus until urine is noted then advance 1-3 additional N.B: If no urine appears (catheter in vagina), leave it as a landmark, re-clean, and insert a new sterile catheter.			
Hold the catheter securely with the non-dominant hand and check for urine flow before inflating the balloon.			
Gently pull the catheter until resistance is felt, then			

advance it slightly to prevent irritation.			
Collect urine specimen (if required): Pinch the catheter, release into a sterile container, then secure and label the container.			
Inflate the balloon by injecting the required amount of sterile water (usually 10ml), ensuring no pain is felt by the patient.			
Attach drainage to collecting tube of drainage bag			
Attach drainage bag to bed frame with hook or strap provided.			
Anchor the catheter using non allergic tape. Tape to inside the thigh and allow some in catheter length.			
Coil excessive drainage tubing and place beside the client, the remaining tubing should run in a vertical line down to the bottom sheet with tape or clip provided with drainage set.			
Place drainage bag below level of bladder.			
Dispose of gloves, equipment and wash hands.			
Help patient adjust position.			
Evaluate catheter function and amount, color, odor and other urine characteristics.			
Document size of catheter inserted, amount of water in ballon, patient tolerance, and assessment of urine.			

If the student doesn't excess 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Enema Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
Verify the Doctor's Order for enema type, volume, and medication (if applicable).			
Wash Hands and Apply Gloves.			
Gather Equipment (enema solution, enema bag or bulb, lubricant, gloves, towels, bedpan or toilet).			
Identify patient & Explain the Procedure to the Patient			
Place the patient in the left lateral decubitus position			
Prepare the Enema Solution: - Warm the enema solution to body temperature (98.6°F or 37°C). - Check the solution's type and concentration.			
Lubricate the Nozzle			
Gently insert the lubricated nozzle into the patient's rectum(about 2-4 inches into the rectum).			
Administer the Enema Solution: - Open the clamp to allow the solution to flow slowly into the rectum and elevate it 12-18 inches above the patient's anus to control the flow			
Observe the patient for signs of discomfort, pain, or cramping during the administration.			
Ask the patient to hold the enema for the prescribed duration (usually 5-15 minutes, depending on the type of enema).			
Assist the patient to expel the solution via a bathroom or a bedpan if they are unable			
Reassessment of patient for Pain and Discomfort			

Evaluate amount, color, odor and other stool characteristics			
Help patient adjust position.			
Proper discard of used equipment according to hospital policy & wash hands			
Document time ,date , type of enema inserted , patient tolerance to procedure, stool characteristics, recommendation and nurse signature			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Heat Application Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Review the medical order for: frequency, type of therapy, body area to be treated and length of time for the application			
2. Prepare Nurse: Perform hand hygiene and put on PPE, if indicated..			
3. Prepare the environment: Close curtains around bed and close room door and window			
4. Prepare Equipment: - Hot water bottle, or aquathermia pad or -heating pad -Bath thermometer for hot water bottle -Flannel cover for hot water bottle			
5. Prepare patient: Identify the patient & Explain procedure to the patient.			
6. Place the patient comfortably with easy access to the target area.			
7. Assess the condition of the skin where the heat is to be applied.			
<u>If using a hot water bottle</u> a- check temperature of the heated tap water with a bath thermometer before filling the hot water bottle. b- Fill the hot water bottle two-thirds full with warm water and remove air from the bottle and close it c-check for leaks d- place it on the affected area e- Keep the bottle in place for 20– 30 minutes			
<u>If using a disposable heat pack.</u> 1. Activate the pack according to the manufacturer's directions. 2. Warp the pack in a towel and place it on the affected area.			

3. Discard the pack after treatment			
<u>If using Electrical heating pad</u> - place the flannel cover on the heating pad. - Do not allow the patient to lie directly on the pad. - Turn the switch on to low and place the heating pad on the affected area - Instruct the patient not to increase the heat level.			
<u>If using an aquathermia pad</u> a- Check that the water in the electronic unit is at the appropriate level. b- Fill the unit two- thirds full or to the fill mark, with distilled water, if necessary. c- Check the temperature setting on the unit to ensure it is within the safe range. d- Attach pad tubing to electronic unit tubing. e- Ensure that there is no leakage. f- Plug in the unit and warm the pad before use. g- Apply the heating pad to the prescribed area & Secure with tape h- Assess skin condition and the patient's response at frequent intervals. i- Do not exceed the prescribed duration			
8. Remove gloves, all remaining equipment & discard			
9. Return the patient to a comfortable position			
10. Reassess the patient and area of application, noting the effect and presence of adverse effects.			
11. Documentation: - Document the rationale for application. If patient is receiving heat therapy for pain, Document the assessment of pain pre- and post- intervention. - Specify the type of heat therapy and location, as well as duration - Document the condition of the skin, noting any redness or irritation before the heat, patient's reaction, any patient or family education.			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature:

Cold Application Checklist

Student name:

Date:

ID:

Steps	Completely Done	Incompletely Done	Not Done
1. Follow steps from 1-7 as heat application.			
2. Choose an appropriate cold source (e.g., ice pack, cold compress, frozen peas, or a cold gel pack).			
3. Position the affected area elevated above the heart level to reduce swelling.			
4. Apply the cold source to the injured area for 15-20 minutes.			
5. Check the skin after the first 5 minutes and every few minutes thereafter.			
6. If the skin becomes too cold, numb, or painful, remove the cold source immediately.			
7. Rest the affected area post application to allow the area to "warm up" post-therapy			
8. Repeat cold application if necessary: Repeat cold therapy every 1-2 hours during the first 48 hours after injury.			
9. Adjust cold application based on comfort: If the cold application is too uncomfortable, reduce the application time or use a less intense cold source (e.g., colder ice pack vs. gel pack).			
10. Follow steps 8-11 as in heat application			

If the student doesn't exceed 70%, the procedure should repeat.

Total grade:

Clinical instructor

Student signature: